

Medical Report — Easy Explanation

An easy-to-understand explanation of your medical report and test results

PATIENT INFORMATION

Patient Name: Johnny S.

Date of Birth: August 14, 1989 (37 yrs)

Sex / MRN: Male | RMF-TEST-20481

Date of Visit: September 24, 2026

Attending Doctor: Dr. Emily Carter, MD

Department: Internal Medicine Clinic

REPORT OVERVIEW

This report explains your outpatient medical follow-up visit on September 24, 2026. The primary focus of this appointment was to evaluate your **blood pressure control**, review your **recent blood and cholesterol lab results**, and address symptoms of **seasonal nasal congestion and heartburn**. Below is a structured, plain-language translation of all medical findings.

KEY HIGHLIGHTS & IMPORTANT FINDINGS

■ ELEVATED	Blood Pressure remains elevated — Clinic reading was 148/92 mmHg, matching home averages of 145–150/90–94 mmHg.
■ HIGH	Blood Sugar results are high — Fasting blood glucose is 156 mg/dL and HbA1c is 7.4% (in the diabetes range).
■ ABNORMAL	Cholesterol panel is elevated — Total cholesterol (232 mg/dL), LDL (152 mg/dL), and triglycerides (205 mg/dL) are above target; HDL (39 mg/dL) is low.
■ NORMAL	Kidney, Liver & Thyroid functions are healthy — Creatinine (1.0 mg/dL), eGFR (94), ALT liver enzyme (29 U/L), and TSH (2.1 mIU/L) are all fully within normal limits.
■ NOTED	Allergies & Reflux noted — Mild nasal swelling seen on physical exam from seasonal allergies, and heartburn symptoms noted.

TEST RESULTS EXPLAINED

Complete breakdown of laboratory blood tests, reference ranges, and plain-language interpretations.

1. Blood Sugar & Glucose Control

Test Name	Your Result	Healthy Target	Status
Fasting Blood Glucose Blood sugar after 8+ hrs fasting	156 mg/dL	70 – 99 mg/dL	HIGH
HbA1c (Hemoglobin A1c) Average 3-month blood sugar	7.4 %	< 5.7% (Healthy) 5.7–6.4% (Pre-diabetes) ≥ 6.5% (Diabetes range)	HIGH (Diabetic)

What this means: HbA1c shows your average blood sugar over the last 2 to 3 months. A result of **7.4%** indicates that average blood sugar is elevated into the diabetes range, meaning your body is having trouble processing glucose efficiently.

2. Cholesterol & Lipid Panel

Lipid Component	Your Result	Healthy Target	Status
Total Cholesterol	232 mg/dL	< 200 mg/dL	HIGH
LDL Cholesterol (Bad fat)	152 mg/dL	< 100 mg/dL (Optimal)	HIGH
HDL Cholesterol (Good fat)	39 mg/dL	≥ 40 mg/dL (Men)	LOW
Triglycerides (Stored fat)	205 mg/dL	< 150 mg/dL	HIGH

What this means: LDL ('bad' cholesterol) can build up along artery walls over time. Higher LDL and triglycerides combined with lower HDL ('good' protective cholesterol) increase cardiovascular risk, which is why your doctor prescribes cholesterol medication and lifestyle changes.

3. Organ Health & Vital Signs

Kidney, Liver & Thyroid Tests		Result & Status	Vital Signs & Measurements	Result & Status
Creatinine (Kidney filter waste) Normal: 0.7 – 1.3 mg/dL		1.0 mg/dL [NORMAL]	Blood Pressure (BP) Target: < 120/80 mmHg	148/92 mmHg [HIGH]
eGFR (Kidney filtration efficiency) Normal: ≥ 60 mL/min/1.73m²		94 [NORMAL]	Body Mass Index (BMI) Normal: 18.5 – 24.9 kg/m²	27.9 kg/m² [OVERWEIGHT]
ALT (Liver enzyme) Normal: 7 – 56 U/L		29 U/L [NORMAL]	Blood Oxygen (SpO2) Normal: 95 – 100%	98% (Room Air) [NORMAL]
TSH (Thyroid hormone) Normal: 0.4 – 4.0 mIU/L		2.1 mIU/L [NORMAL]	Pulse / Temp / Resp Heart rate, temperature, breathing	82 bpm 98.4°F 16/min [ALL NORMAL]

CONDITIONS MENTIONED IN THE REPORT

Every condition identified in your report is clearly classified below by its clinical status (Confirmed, Past History, or Assessment).

■ CONFIRMED CONDITION • ACTIVE DIAGNOSIS

Essential Hypertension (High Blood Pressure)

What it means: The force of blood against your artery walls is consistently higher than it should be (measured at 148/92 mmHg in clinic and 145–150/90–94 mmHg at home). Uncontrolled high blood pressure places extra strain on the heart and arteries over time.

■ CONFIRMED CONDITION • ACTIVE DIAGNOSIS

Mixed Hyperlipidemia (High Cholesterol & Triglycerides)

What it means: You have an unhealthy balance of fats in your blood: high total cholesterol (232 mg/dL), high bad LDL (152 mg/dL), high triglycerides (205 mg/dL), and low good HDL (39 mg/dL).

■ CONFIRMED CONDITION • ACTIVE SYMPTOM

Gastroesophageal Reflux Disease (Heartburn / Acid Reflux)

What it means: Stomach acid backs up into the esophagus (food pipe), causing burning chest discomfort and a sour taste, especially after heavy meals.

■ CONFIRMED CONDITION • ACTIVE SYMPTOM

Allergic Rhinitis (Seasonal Allergies / Nasal Congestion)

What it means: Seasonal allergies causing sneezing, runny nose, and mild swelling inside the nasal passages.

■ FROM PAST MEDICAL HISTORY & CLINICAL ASSESSMENT

Prediabetes & Blood Sugar Elevation (Type 2 Diabetes Criteria)

What it means: Prediabetes was previously documented in your medical history. Your current lab results (Fasting Glucose 156 mg/dL and HbA1c 7.4%) indicate progression into the Type 2 Diabetes range. Your doctor has you on blood sugar medication (metformin) and scheduled repeat testing in 3 months.

MEDICATIONS SUMMARY

The medications mentioned in your visit summary, including their dosages, schedules, and specific purposes.

■ CURRENT MEDICATION

Amlodipine — 10 mg once daily (Increased from 5 mg)

Prescribed For: Blood pressure control

How it works: Relaxes and widens your blood vessels so blood can flow more easily, lowering blood pressure. Your doctor increased the dose to help reach your target pressure.

■ CURRENT MEDICATION

Metformin — 500 mg twice daily with meals

Prescribed For: Blood sugar management

How it works: Helps your body use insulin more effectively and reduces the amount of glucose produced by your liver. Taking it with meals reduces stomach upset.

■ CURRENT MEDICATION

Atorvastatin — 20 mg once daily at bedtime

Prescribed For: Cholesterol & cardiovascular protection

How it works: A statin medication that lowers "bad" LDL cholesterol and triglycerides while stabilizing blood vessel walls.

■ CURRENT MEDICATION

Omeprazole — 20 mg once daily before breakfast (as needed)

Prescribed For: Stomach acid & heartburn relief

How it works: Reduces the amount of acid your stomach produces to soothe the esophagus and prevent heartburn.

Important Note: Never change, stop, or skip your prescribed medication without discussing it first with your doctor or pharmacist.

WHAT THE REPORT DID NOT FIND

These critical conditions and symptoms were explicitly evaluated during your visit and were **ruled out / not present**. This provides reassuring context about your overall health.

✓ **No Evidence of Heart Attack (Myocardial Infarction)**
You had no severe chest pain, left arm pain, or pressure, and your cardiovascular evaluation was reassuring.

✓ **No Evidence of Stroke**
Your neurological examination was completely normal with no facial drooping, focal muscle weakness, or speech issues.

✓ **No Evidence of Kidney Disease**
Your kidney blood tests (Creatinine 1.0 mg/dL and eGFR 94) confirm that both kidneys are filtering waste normally.

✓ **No Evidence of Edema (Fluid Swelling)**
Physical examination showed no swelling in your lower legs, ankles, or feet.

✓ **No Shortness of Breath or Breathing Difficulty**
Your lungs were completely clear upon examination and oxygen levels were optimal at 98%.

✓ **No Fever, Cough, or Acute Infection**
There were no signs of respiratory tract infection, pneumonia, or systemic illness.

Additional Symptoms Evaluated and Denied: Chest pain, syncope (fainting), palpitations (rapid heart beats), vomiting, abdominal pain, excessive urination (polyuria), wheezing, and unexplained weight loss were all confirmed **absent**.

MEDICAL TERMS EXPLAINED

Medical reports often use technical clinical terms. Here are simple, everyday translations of the terms used in your report.

EPIGASTRIC DISCOMFORT

In simple terms: Burning, fullness, or pain felt in the upper-middle area of your abdomen right below the ribcage (commonly known as heartburn or indigestion).

NASAL MUCOSAL EDEMA

In simple terms: Mild swelling of the delicate, pink tissue lining the inside of your nose, typically triggered by seasonal airborne allergies.

CLEAR TO AUSCULTATION (LUNGS CTA)

In simple terms: When the physician listened to your chest and back using a stethoscope, normal and healthy airflow was heard with no wheezing or crackles.

NON-DISTENDED, NON-TENDER ABDOMEN

In simple terms: Your belly area was soft, not swollen or bloated, and did not hurt when the doctor pressed gently on it.

GROSSLY INTACT NEUROLOGICAL EXAM

In simple terms: Your nerve function, body reflexes, muscle strength, balance, and sensations across arms and legs are completely normal.

FASTING BLOOD GLUCOSE VS. HBA1C

In simple terms: Fasting glucose measures your blood sugar at one exact moment in the morning; HbA1c measures your average blood sugar level over the past 90 days.

FOLLOW-UP & ACTION PLAN

Important next steps, home monitoring instructions, repeat laboratory tests, and upcoming appointments scheduled by your healthcare provider.

■ 1. Home Blood Pressure Monitoring

Record your blood pressure at home **twice daily** (morning and evening) for the next **14 days**. Bring your written log or phone app log to your next clinic appointment.

■ 2. Continue Prescribed Medication Routine

Take your adjusted **Amlodipine (10 mg)** once daily and continue **Metformin** twice daily with meals. Take Atorvastatin at bedtime.

■ 3. Dietary & Lifestyle Adjustments

Emphasize a heart-healthy, lower-sodium diet. Avoid late-night heavy meals and spicy/acidic foods that trigger heartburn.

■ 4. Repeat Laboratory Testing in 3 Months

Schedule repeat blood tests for **HbA1c** (blood sugar) and a **Lipid Panel** (cholesterol) in approximately 3 months to monitor medication effectiveness.

■ 5. Clinic Follow-up Visit in 4 Weeks

Return to the Internal Medicine clinic in **4 weeks** with Dr. Emily Carter to review your home blood pressure log and discuss your response to the adjusted medication dose.

Important Medical Disclaimer: This document is an AI-generated plain-language explanation of the uploaded medical report. It is intended solely to help make clinical information easier to understand and does not replace medical advice, diagnosis, or treatment from a qualified healthcare professional. If you experience severe symptoms, chest pain, or shortness of breath, seek immediate medical attention.